Title: Intersectionality, Reproductive Rights, and the State: A Comparative Legal Analysis of Surrogacy Access for Bisexual Men with Complex PTSD in the United Kingdom and Ireland

1. Introduction: The Evolution of Non-Traditional Fatherhood in the British Isles

The legal landscape governing assisted human reproduction (AHR) and surrogacy in the British Isles is currently characterized by a profound and widening divergence. On one side of the Irish Sea, the United Kingdom operates under a mature, albeit statutorily aging, framework that has been progressively modernized through judicial activism, remedial orders, and a pragmatic regulatory culture to include single and LGBTQ+ parents. Conversely, the Republic of Ireland is in the throes of a radical legislative overhaul with the *Health (Assisted Human Reproduction) Act 2024*, a piece of legislation that promises modernization but is currently beset by implementation delays, bureaucratic inertia, and supranational legal conflicts.

This report provides an exhaustive analysis of the specific legalities facing a singular, intersectional applicant profile: a **single, bisexual man with a diagnosis of Complex Post-Traumatic Stress Disorder (C-PTSD)** who wishes to conceive a child via In Vitro Fertilisation (IVF) and surrogacy. This applicant profile presents a unique constellation of legal challenges that tests the boundaries of family law in both jurisdictions. It requires navigating laws historically designed for heterosexual couples, overcoming the "legal invisibility" of bisexuality in family courts, and subjecting one's mental health history to state scrutiny under "Welfare of the Child" (UK) or "Suitability" (Ireland) provisions.

The comparative analysis reveals that while the United Kingdom has moved towards a risk-based model that largely facilitates reproductive autonomy for managed mental health conditions, Ireland remains trapped in a "suitability" paradigm derived from adoption law. This Irish model, exacerbated by the recent conflict between domestic legislation and the European Union's *Anti-Trafficking Directive 2024/1712*, creates a precarious environment for applicants with complex personal histories. The following analysis dissects these frameworks, contrasting the "modern approach" of the UK—defined by judicial pragmatism—with the "lack of progress" in Ireland, defined by legislative paralysis.

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2. The Applicant Profile: Intersectionality in

Reproductive Law

To understand the legal hurdles, one must first deconstruct the intersectional identity of the applicant. In family law, identity is not merely descriptive; it is determinative. The interaction between being a single male, a bisexual individual, and a person managing C-PTSD creates a specific matrix of risk in both the UK and Irish legal systems.

2.1. The Single Father: Deconstructing the "Need for a Mother"

Historically, reproductive law in both jurisdictions was designed around the heterosexual dyad. The "need for a father" clause in the UK's original *Human Fertilisation and Embryology Act 1990* was a statutory manifestation of this bias, explicitly requiring clinics to consider a child's need for a father figure before providing treatment. This was repealed and replaced with "supportive parenting" in 2008, signaling a shift toward recognizing diverse family structures.[1] For the single male applicant, the ability to act as the sole legal parent is the foundational threshold. If the law does not recognize the single man as a potential parent *ab initio*, the questions of bisexuality or mental health become moot.

- **In the UK**, the decisive shift occurred with the *Human Fertilisation and Embryology Act 2008 (Remedial) Order 2019*. This legislation was a direct response to the declaration of incompatibility in *Re Z (A Child) (No 2) *, where the High Court found that barring a single father from obtaining a Parental Order violated Article 14 (non-discrimination) of the European Convention on Human Rights (ECHR).[2] Consequently, a single man in the UK today stands on equal statutory footing with couples regarding eligibility for a Parental Order, provided he provides his own gametes.[4]
- **In Ireland**, the legal recognition of the single father has been slower and more convoluted. The *Health (Assisted Human Reproduction) Act 2024* explicitly contemplates the "single intending parent" in Section 56.[5] This inclusion is significant in a country where the constitutional definition of "family" was traditionally tethered to marriage. However, the legislation introduces a curious gender paradox. Section 56(3)(c) requires intending parents to demonstrate a medical necessity for surrogacy (e.g., inability to gestate). For a single man, this inability is biological and assumed; for a single woman, it must be medically proven (e.g., absence of a uterus).[6] While this paradox technically benefits the male applicant, it highlights the gendered and essentialist underpinnings of the Act that could manifest negatively in other areas, such as suitability assessments.

2.2. Bisexuality: The Invisible Orientation in Family Court

While sexual orientation is a protected characteristic under the UK's *Equality Act 2010* and Ireland's *Equal Status Acts*, bisexuality often suffers from "legal invisibility" or "monosexism" in family law contexts. In the binary world of "gay" vs. "straight" adoption and surrogacy applicants, bisexual men often face a unique set of prejudices.

Research indicates that bisexual parents often fear their sexual identity will be conflated with instability, promiscuity, or a "lack of commitment," factors explicitly weighed in child welfare

assessments.[7] In the context of the UK's HFEA assessment or Ireland's AHRRA suitability test, a bisexual man's relationship history—which may include partners of different genders—can be misinterpreted by conservative assessors as evidence of a "chaotic lifestyle" rather than a valid sexual identity.[8] This "erasure" often forces bisexual applicants to "closet" themselves, presenting as either "gay" or "single/celibate" to fit the assessor's template, a strategy that introduces the risk of deception in a legal process that demands full disclosure.[9]

2.3. Complex PTSD: The Medical-Legal Interface

Complex Post-Traumatic Stress Disorder (C-PTSD) is a condition distinct from standard PTSD, arising from prolonged, repeated trauma (often interpersonal, such as childhood abuse or domestic violence). It is characterized by three additional symptom clusters beyond standard PTSD:

1. **Affect Dysregulation:** Difficulty managing intense emotions.
2. **Negative Self-Concept:** Feelings of shame, guilt, or worthlessness.
3. **Interpersonal Disturbances:** Difficulty sustaining relationships or feeling close to others.[10]

In the context of reproductive law, C-PTSD is the primary variable that triggers "Welfare of the Child" scrutiny. The legal question is not whether the applicant *has* a condition, but whether that condition poses a "risk of serious harm" (UK) or renders the applicant "unsuitable" (Ireland).

- **The Risk Narrative:** Legal frameworks for child welfare are inherently risk-averse. Symptoms like "affect dysregulation" can be terrifying to a social worker or family court judge, who may interpret them as a potential for volatile parenting.[11]
- **The Trauma Narrative:** Conversely, modern "trauma-informed" care suggests that managed C-PTSD can equate to high resilience and empathy. The divergence between the UK and Ireland lies in how much space the legal system creates for this "resilience" narrative versus the "risk" narrative.[12]

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3. The United Kingdom: A Mature Regulatory State

The United Kingdom's approach to surrogacy and IVF is characterized by a "modern approach" that prioritizes the welfare of the child while increasingly respecting the autonomy of the intended parent. This modernity is not necessarily found in the text of the primary legislation—the *Surrogacy Arrangements Act 1985* is widely considered archaic—but in the regulatory culture of the Human Fertilisation and Embryology Authority (HFEA) and the pragmatic jurisprudence of the Family Division of the High Court.

3.1. The Statutory Framework: From 1985 to 2019

The UK operates under a "compensatory but altruistic" model. Commercial surrogacy is prohibited domestically, but the law has evolved to accommodate the reality of modern family building.

3.1.1. The Parental Order (PO): Section 54 and 54A

The Parental Order is the transformative legal mechanism that extinguishes the surrogate's legal parenthood and transfers it to the Intended Parent (IP). Without a PO, the biological father has no legal decision-making power over the child regarding education or medical care, and the child remains the legal responsibility of the surrogate.[13]

The critical development for the bisexual single male applicant was the *Human Fertilisation and Embryology Act 2008 (Remedial) Order 2019*. Prior to this, Section 54 of the 2008 Act restricted POs to "two people." This excluded single parents, forcing them to rely on adoption—a process that severed the legal link to the surrogate but did not recognize the biological link of the father in the same way. The 2019 Order introduced **Section 54A**, which allows a single applicant to apply for a PO if:

- **Genetic Link:** The applicant is the biological father (sperm provider).[4]
- **Domicile:** The applicant is domiciled in the UK, Channel Islands, or Isle of Man.
- **Residence:** The child lives with the applicant at the time of application.[4]
- **Time Limit:** Application is made within 6 months of birth, though courts routinely extend this in the child's best interests (e.g., *Re X *).[15]

This legislative change was a watershed moment, placing single men on an equal statutory footing with couples. It signaled that the state no longer viewed the "single status" as an inherent defect in parenting capacity.[2]

3.1.2. The Role of CAFCASS

In every Parental Order application, a Parental Order Reporter from the Children and Family Court Advisory and Support Service (CAFCASS) is appointed. Their role is to act as the eyes and ears of the court, investigating the circumstances of the surrogacy and the welfare of the child.[13]

For an applicant with C-PTSD, the CAFCASS reporter is the primary point of scrutiny. The reporter will interview the applicant, observe them with the child, and assess the home environment. Critically, CAFCASS operates under the "welfare checklist" of the *Adoption and Children Act 2002* (applied to POs). While they assess risk, the UK family courts have established a strong presumption in favor of making the order. The guiding principle is that leaving a child legally "limping"—without the legal parenthood of their biological caregiver—is almost never in the child's best interests.[16] Rejections of POs on welfare grounds are exceptionally rare and typically involve egregious circumstances (e.g., active, severe abuse), not managed mental health conditions like C-PTSD.[17]

3.2. The HFEA "Welfare of the Child" Assessment: A "Negative Right" Model

Before a clinic in the UK can provide IVF services (creating the embryo with the applicant's sperm and donor eggs), they must complete a "Welfare of the Child" (WOC) assessment under Section 13(5) of the HFE Act 1990. This is the first gatekeeper.

3.2.1. The Shift from "Suitability" to "Risk of Harm"

Historically, this assessment was intrusive and moralistic. However, the HFEA Code of Practice (8th and 9th Editions) explicitly moved away from judging "parenting ability" or social conformity. It shifted the focus to a "harm threshold." The current test is: **Is there evidence that the child will suffer *serious* medical, physical, or psychological harm?**.[1]
This is a "negative right" model. It assumes the applicant has a right to treatment *unless* the state (via the clinic) can prove a risk of serious harm. It does not require the applicant to prove they are a "super parent."

3.2.2. Application to C-PTSD

For the applicant with C-PTSD, this distinction is vital.

- **The Process:** Clinics mandate a "Welfare of the Child" form.[19] Question 2.4 asks: "Do you have any mental or physical conditions?"
- **Disclosure:** If the bisexual man discloses C-PTSD, the clinic is obligated to investigate further. However, they are guided *not* to refuse treatment solely on the diagnosis.
- **The "Managed Condition" Standard:** The clinic will typically request a letter from the applicant's GP or treating psychiatrist/psychotherapist. If the medical professional confirms that the C-PTSD is "managed," that the applicant has "insight" into their triggers, and that there is no history of violence or neglect, the assessment is usually satisfied.[20]
- **The Support Network:** For single applicants, the HFEA guidance emphasizes the "support network." A single father with C-PTSD will be asked, "Who supports you when you are triggered or having a bad day?" A robust answer (e.g., involved family, close friends) effectively mitigates the perceived risk.[2]

This system is designed to facilitate treatment. Clinics are private entities that wish to provide services, and they are wary of the *Equality Act 2010*, which prohibits discrimination against disabled persons (including those with long-term mental health conditions) unless it is a proportionate means of achieving a legitimate aim (child safety). The burden of proof lies on the clinic to show why C-PTSD makes the applicant dangerous, a burden that is difficult to meet if the applicant is high-functioning.[18]

3.3. Stalled Reforms: The Law Commission Report 2023 and the 2025 Rejection

Despite the "modern approach" of the HFEA and the courts, the underlying statutory framework (1985 Act) remains outdated. In March 2023, the Law Commission of England and Wales proposed a comprehensive reform package.

- **The "New Pathway":** The core proposal was a new pathway for domestic surrogacy where the Intended Parents would become the legal parents *at birth*, bypassing the court process entirely, provided specific pre-conception screening was undertaken.[22]
- **The Rejection:** In April 2025, the UK Government formally responded, confirming that it would *not* prioritize these reforms in the current parliamentary session due to

"pressures on parliamentary time".[23]

Implication for the Applicant: This "lack of legislative progress" in the UK means the applicant must still navigate the clumsy post-birth Parental Order process. For a man with C-PTSD, this period of "limbo" (6-12 months where the surrogate is the legal mother) can be a significant source of anxiety, potentially exacerbating symptoms of hypervigilance. However, the system is *predictable*. The courts have never refused a PO to a biological father based on a managed mental health condition, providing a high degree of legal certainty despite the procedural archaicness.

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4. Ireland: A System in Transition, Stagnation, and Conflict

In stark contrast to the UK's stable but aging system, Ireland is in a state of legislative flux. The enactment of the *Health (Assisted Human Reproduction) Act 2024* was intended to be a modernizing force, finally regulating a sector that had operated in a legal vacuum. However, as of 2026, the implementation of this Act has stalled, creating a "no-man's-land" that is arguably more perilous for the applicant than the previous unregulated regime.

4.1. The Pre-2024 Vacuum and Constitutional Context

Prior to the 2024 Act, surrogacy in Ireland was entirely unregulated. Under the principle of *mater semper certa est* ("the mother is always certain"), the woman who gave birth (the surrogate) was the legal mother, regardless of genetics. The biological father could only apply for guardianship, a legal status inferior to full parentage that expires when the child turns 18.[26]

- **The "Family" Definition:** Article 41 of the Irish Constitution affords special protection to the "Family," historically interpreted as the family based on marriage. While the 39th Amendment (2024) sought to expand this to "durable relationships," the rejection of that referendum left the definition somewhat traditional, though the *Children and Family Relationships Act 2015* had already broadened statutory recognition of diverse families. Nevertheless, the constitutional shadow often creates a conservative baseline for "best interests" assessments.[9]

4.2. The Health (Assisted Human Reproduction) Act 2024: Promise vs. Reality

The 2024 Act introduces a regulated framework for both domestic and international surrogacy.

- **Domestic Surrogacy (Part 7):** Allows for a "pre-conception" approval by the new regulatory body, the **Assisted Human Reproduction Regulatory Authority (AHRRA)**. If approved, a Parental Order can be granted at birth (or shortly thereafter), making the Intended Parent the legal parent immediately.
- **International Surrogacy (Part 8):** Allows for retrospective recognition of parentage for

arrangements abroad, provided they meet strict criteria (the "green list" of approved jurisdictions) and involved only "reasonable expenses".[28]

Section 56 and the Single Father: As noted in Section 2.1, the Act permits single men to apply. This is a progressive step. However, the Act requires the AHRRA to be satisfied of the applicant's "suitability" *before* conception (for domestic) or before recognition (for international).[30]

4.3. The Implementation Crisis (2025-2026)

Despite the Act's passage in July 2024, the operational reality in 2026 is one of paralysis.

- **AHRRA Establishment:** The AHRRA was formally "established" in October 2025, but it is not yet fully operational.[31] It lacks the published Codes of Practice necessary to process applications.
- **Non-Commencement:** The Minister for Health has delayed the commencement of the specific sections allowing for Parental Orders and international recognition.
- **The "No-Man's-Land":** This delay traps current applicants. An Irish bisexual man cannot access the new "pre-conception" approval because the AHRRA isn't ready to give it. Yet, he cannot confidently rely on the old "unregulated" model because the new Act contains retrospective provisions that might penalize arrangements entered into during this interim period if they don't comply with rules that haven't been fully published yet.[33]

4.4. The EU Anti-Trafficking Directive Conflict

The primary driver of the delay in Ireland—and a unique risk for the Irish applicant—is the conflict with the European Union.

- **The Directive:** In June 2024, the EU adopted *Directive 2024/1712*, amending the 2011 Anti-Trafficking Directive. Crucially, Article 2 was amended to include **"exploitation of surrogacy"** as a form of human trafficking.[35]
- **The Ambiguity:** The Directive does not define "exploitation" merely as "coercion." Abolitionist feminist groups and conservative NGOs have successfully lobbied for an interpretation where *any* payment beyond reasonable expenses (i.e., commercial surrogacy) constitutes exploitation of the surrogate's reproductive body.[37]
- **The Irish Dilemma:** Ireland intended to recognize international surrogacy from countries like the USA (where commercial payments are standard). However, the Department of Justice and the Attorney General have advised that recognizing such arrangements might now constitute "facilitating human trafficking" under the new EU law. Ireland must transpose this Directive by July 2026.[39]
- **The Impact:** This has caused a freeze on the "International Surrogacy" provisions of the 2024 Act. The government is terrified of an infraction procedure from the European Commission.
- **Contrast with UK:** The UK, post-Brexit, is *not* bound by this Directive. It continues to recognize US commercial surrogacy arrangements via the High Court's jurisdiction, prioritizing the child's welfare over public policy concerns regarding payment. The Irish

applicant is thus uniquely disadvantaged by Ireland's EU membership in this specific context.

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5. The "Suitability" Trap: A Deep Dive into Assessment Methodologies

For the bisexual applicant with C-PTSD, the difference between the UK's "Welfare" assessment and Ireland's "Suitability" assessment is the difference between a hurdle and a wall.

5.1. United Kingdom: The Presumption of Competence

As detailed in Section 3.2, the UK model is a **Risk Assessment**.

- **Philosophy:** The state intervenes only to prevent *harm*.
- **Mechanism:** The "GP Letter."
- **C-PTSD Handling:** The focus is on *management*. If the applicant is in therapy and functioning, the risk is deemed low. The system accepts that parents can have mental health histories. The HFEA guidelines explicitly state that a history of mental illness is not a bar to treatment.[18]
- **Outcome:** High certainty. The applicant can know, pre-conception, that he will be treated.

5.2. Ireland: The "Best Interests" and Adoption Standards

The 2024 Act requires the AHRRA to determine "suitability." In the absence of established AHR precedents, the AHRRA Board (comprising bioethicists and legal scholars, some with conservative track records [32]) is expected to borrow heavily from the **Adoption Authority of Ireland (AAI)** standards.

- **Philosophy:** The state intervenes to ensure the *best possible* outcome for the child. This is a **Positive Vetting** model. It asks not "are you dangerous?" but "are you good enough?".[40]
- **Adoption Assessment Mechanism:**
  - **The "Home Study":** A rigorous process involving multiple interviews, home visits, and "Life Story" analysis.[41]
  - **Trauma Resolution:** Irish adoption social work places a heavy emphasis on "resolved trauma." Applicants are often expected to demonstrate that they have fully "processed" and "moved past" their childhood trauma.[12]
  - **The C-PTSD Catch-22:** C-PTSD is, by definition, a chronic condition of managing trauma responses. An assessor using an adoption framework might view the ongoing symptoms of C-PTSD (e.g., occasional dissociation or need for therapy) as evidence that the trauma is "unresolved." This could lead to a finding of unsuitability, or a requirement for years of "stabilization" before approval is

granted.[43]

- **Case Law Precedent:** In cases like *Adoption Authority of Ireland v C & D* and *Re B*, Irish courts have demonstrated a willingness to override the wishes of biological parents or foster parents if they believe the "best interests" (often interpreted conservatively regarding stability and mental health) are not met.[44]

5.3. Bisexuality in the Assessment Room

In the UK, the HFEA assessment is largely a paper exercise. In Ireland, the "suitability" assessment is narrative.

- **Implicit Bias:** A bisexual man discussing his romantic history in a deep-dive interview faces the risk of "monosexism." An assessor trained in traditional family structures might view a history of relationships with both men and women as "confusion" or "instability," rather than a valid orientation.
- **Data:** Research from the *Northern Ireland Life and Times Survey* and other Irish studies suggests that while overt homophobia has decreased, subtle bias against "non-normative" family structures persists in social work and family law.[9] Bisexual men often report feeling the need to "sanitize" their history to appear "stable" to assessors, creating a psychological burden that exacerbates C-PTSD symptoms (shame/concealment).[7]

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6. Detailed Comparative Analysis: UK vs. Ireland

6.1. Table 1: Regulatory Comparison for Single Bisexual Men with C-PTSD

Feature	United Kingdom (UK)	Republic of Ireland (ROI)
Legislation	*Surrogacy Arrangements Act 1985*; *HFEA 2008* (Remedial Order 2019)	*Health (AHR) Act 2024* (Enacted but largely uncommenced)
Single Male Eligibility	**Yes** (Section 54A). No medical proof needed.	**Yes** (Section 56). No medical proof needed (unlike women).
Bisexuality Protection	*Equality Act 2010*. High legal protection in service provision.	*Equal Status Acts*. High legal protection, but cultural bias persists in "suitability" tests.
Mental Health Threshold	**"Risk of Serious Harm"** (HFEA). High threshold for refusal. Focus on safety.	**"Suitability"** (AHRRA). Likely "Best Interests" model. Focus on parenting capacity/resolved trauma.
Assessment Body	Fertility Clinics (Delegated by HFEA).	AHRRA (Assisted Human Reproduction Regulatory Authority).

Surrogacy Model	Altruistic only. Unenforceable contracts.	Altruistic (Domestic). International "Green List" (Commercial permitted if retrospective - arguably).
International Surrogacy	**Permitted via Parental Order.** Courts prioritize child welfare over public policy breaches (payments).	**Heavily Regulated/Stalled.** New Act requires pre-approval. Conflict with EU Trafficking Directive causing delays.
Transfer of Parentage	Post-birth Parental Order (6-12 months).	Domestic: Pre-conception approval $\rightarrow$ PO at birth. International: Post-birth court order (Frozen).
Reform Status	2023 Reforms **Shelved** in 2025. Status quo prevails.	2024 Act **Enacted but Stalled**. Implementation crisis.
Trafficking Directive	**Not Applicable** (Brexit).	**Binding.** Must transpose by July 2026; restricts commercial surrogacy recognition.

6.2. The "Modern Approach" vs. "Lack of Progress"

The user query asks to contrast the "lack of progress in Ireland" with the "more modern approach" in the UK.

- **UK Modernity:** The UK's modernity is not legislative; the 1985 Act is a relic. Its modernity lies in its **judicial pragmatism**. Courts have consistently interpreted the law to reflect modern realities, creating a "functional fiction" where surrogacy works despite the law. The HFEA regulates with a light touch, respecting reproductive autonomy.
- **Irish Stagnation:** Ireland's "lack of progress" is a result of **legislative ambition colliding with administrative paralysis**. The 2024 Act is *too* ambitious—attempting to regulate every aspect of AHR from scratch while simultaneously navigating complex EU obligations. The result is that a perfect system is being built on paper, while real people are left in a legal void.

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7. Strategic Outlook and Conclusions

For a single, bisexual man with C-PTSD, the choice of jurisdiction is determinative of the level of legal risk and psychological stress he will face.

7.1. The UK Pathway: Certainty Through Pragmatism

The UK offers a path of **high certainty**.

- **Legality:** His right to apply for a Parental Order is statutory and secure (Section 54A).

- **Mental Health:** The HFEA "Welfare" check is a hurdle, not a wall. With a supportive GP letter, he will pass.
- **Surrogacy:** He can access international surrogacy (e.g., USA) and be confident that the UK High Court will grant him parentage, ignoring the "commercial" nature of the payments in the best interests of the child.

7.2. The Irish Pathway: Risk Through Uncertainty

Ireland offers a path of **high risk**.

- **Legality:** His right exists on paper but is currently inoperable due to non-commencement.
- **Mental Health:** He faces an AHRRA "suitability" assessment that is likely to be intrusive, deficit-focused, and modeled on adoption standards that penalize "unresolved" trauma like C-PTSD.
- **Surrogacy:** He is currently blocked from "approved" international surrogacy due to the EU Trafficking Directive conflict. If he proceeds now, he risks his child being left in a legal limbo if the Irish state later decides his arrangement constituted "exploitation" under the new EU rules.

7.3. Final Insight

The applicant's specific intersectional profile makes the **United Kingdom** the vastly superior jurisdiction for his family-building journey. The UK system treats him as a citizen with a right to reproduce, limited only by proven danger. The Irish system, in its current unformed state, treats him as a subject who must prove his worthiness to a state apparatus that is itself paralyzed by conflicting definitions of morality and legality. The "lack of progress" in Ireland is not merely a delay; it is a structural failure to reconcile domestic modernization with supranational constraints, leaving vulnerable applicants to bear the cost of the state's indecision.

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